

Acute GI Bleeding — Upper & Lower

Resuscitate first · restrictive transfusion (Hb 7) · localize upper vs lower · risk-stratify with Glasgow-Blatchford · PPI + timely endoscopy for UGIB · treat the variceal bleed differently

1 · Resuscitate & localize

Stabilize before you scope. Two large-bore IVs, type & crossmatch, fluids; assess severity by hemodynamics, not the initial Hb (which lags).

Clue	Points to
Hematemesis / coffee-ground	Upper (above ligament of Treitz)
Melena	Usually upper (sometimes right colon/SB)
BUN:Cr > 30 (no renal cause)	Upper source
Hematochezia + unstable	Brisk upper bleed until proven otherwise
Hematochezia + stable	Lower (diverticular, angioectasia, etc.)

A normal initial Hb does not exclude a major bleed — equilibration takes hours. Trend it and resuscitate to vitals.

2 · Transfuse restrictively

- **Restrictive threshold Hb 7 g/dL** (target 7–9) — better survival than liberal in most UGIB (Villanueva).
- Use a higher threshold (~8) with acute coronary syndrome or significant cardiovascular disease.
- Correct coagulopathy judiciously; **do NOT over-transfuse platelets/plasma** in variceal bleeding — raising portal pressure worsens it.
- Reverse/hold anticoagulants per agent; don't delay endoscopy for a perfect INR.

Over-transfusion raises portal pressure and rebleeding in varices; aim for Hb 7, not a 'normal' number.

3 · Risk-stratify & pre-endoscopy meds

Glasgow-Blatchford score (GBS) at presentation. **GBS 0–1 = very low risk** → consider discharge with outpatient endoscopy.

Drug	Indication
IV PPI	Suspected ulcer/UGIB — start pre-endoscopy
IV erythromycin 250 mg	~20–90 min pre-scope — clears stomach, improves view
Octreotide + antibiotics	Suspected variceal bleed (see box)

Endoscopy timing: within 24 h for most UGIB after resuscitation. *Urgent* <12 h does not improve outcomes and can worsen them — resuscitate first.

Variceal bleed — the different pathway

- Suspect with cirrhosis/portal hypertension. **Start octreotide** (lowers portal pressure) and **prophylactic antibiotics** (ceftriaxone) — antibiotics reduce mortality.
- **Endoscopic band ligation** is first-line; balloon tamponade or TIPS as rescue.
- Restrictive transfusion is especially important — avoid raising portal pressure.

Lower GI bleed & Do/Avoid

- Most LGIB stops spontaneously; common causes: diverticulosis, angioectasia, post-polypectomy, ischemic/IBD colitis, hemorrhoids.
- Colonoscopy after prep for stable patients; **CT angiography** ± embolization for ongoing brisk bleeding; consider upper source if unstable.
- **DO** resuscitate and risk-stratify (GBS) before deciding disposition
- **DO** give pre-endoscopy IV PPI for suspected ulcers and octreotide+antibiotics for varices
- **AVOID** liberal transfusion (Hb target stays 7–9 in most)
- **AVOID** 'emergent' <12 h endoscopy in an under-resuscitated patient
- **AVOID** withholding endoscopy to chase a normal INR/platelet count